National Cancer Control Programme (NCCP) 00569 · clinician draft

# Pembrolizumab + pemetrexed + cisplatin

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Induction: cisplatin-containing cycles 1–4

**Active medicines:** pembrolizumab + pemetrexed + cisplatin

**Emetogenic risk:** high

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg orally Day 1; 80 mg orally Days 2–3 | Before treatment Day 1, then daily Days 2–3 |
| Ondansetron | 16 mg orally | Day 1<br>Individualise for QT risk and patient factors. |
| Antiemetic dexamethasone reference | 12 mg orally Day 1; 8 mg orally daily Days 2–4 | NCCP NCIS V8 high-risk general row<br>Reconcile with pemetrexed rash steroid and aprepitant interaction; issue one clear prescription, do not add overlapping courses. |
| Olanzapine | 5 mg orally daily | Days 1–4<br>Cisplatin high-risk general HEC row. Sedation/metabolic cautions; consultant may individualise. |
| Metoclopramide | 10 mg orally three times daily as prescribed | Subsequent days<br>NCIS table does not specify an end day here; prescribe duration and suitability. Concomitant olanzapine increases extrapyramidal adverse-effect risk. |
| Folic acid | 350–1000 micrograms orally daily | At least five doses in seven days before first pemetrexed; throughout treatment and 21 days after the final dose |
| Vitamin B12 (hydroxocobalamin) | 1000 micrograms intramuscular (IM) | Week before first pemetrexed and every three cycles thereafter<br>Later doses may be on the pemetrexed day. |
| Pemetrexed rash prophylaxis: dexamethasone equivalent | 4 mg orally twice daily | Day before, day of and day after pemetrexed<br>This is the regimen rash schedule; prescriber/pharmacy must reconcile it with antiemetic dexamethasone into one unambiguous prescription, including aprepitant interaction. Do not simply add both courses. |
| Cisplatin hydration and electrolytes | Local individual prescription | Before and after cisplatin<br>NCCP suggested framework: pre 1 L sodium chloride 0.9% over 60 minutes; post 1 L over 2 hours with 10 mmol magnesium sulfate and 20 mmol potassium chloride. Local policy and fluid/electrolyte status govern. Mannitol optional; routine loop diuretic is not required unless fluid overload. |

### Additional phase-specific support

- Cisplatin requires high-risk antiemetics and individual hydration/electrolyte review.
- Folic acid and B12 must be in place before pemetrexed and continue into maintenance.
- Combine rash and antiemetic dexamethasone directions into one reconciled prescription; do not stack courses.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00569 v5 · pNCCP00569 pages 1–2,6; NCIS V8 Table 1 · NCCP NCIS Medical Oncology antiemetics V8

Sources checked 2026-10-29 · SACTCheck v0.84.27 · Regimen-level drafts; prescriber and oncology-pharmacy verification required